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PRODUCT REQUIREMENTS DOCUMENT

OpenLoop-Style Telemedicine Platform

Find-a-Doctor & Open-Loop Care Model

REF NO.	DATE	VERSION	PREPARED FOR
HW-PRD-PRM-2026-001	16 June 2026	1.0	Dr. Param

PREPARED BY

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PREPARED FOR

Dr. Param
Family Physician Clinic
United States
Classification: **Confidential**

01 EXECUTIVE SUMMARY

Dr. Param is building a direct-to-consumer telemedicine platform modelled on Sesame, Zocdoc, and Klarity, with the clinical and operational backbone of an "open-loop" provider network similar to OpenLoop Health. The platform lets a patient discover a service, complete a guided intake, get matched with a licensed clinician, receive care by video or asynchronous review, and leave with a prescription, lab order, or care plan — while a separate clinician workspace and an internal operations console run everything behind the scenes.

The platform is three connected products — the **Patient App**, the **Provider Dashboard**, and the **Admin Console** — released across three versions so the client can launch fast, prove the model with real patients and providers, and only invest in heavier infrastructure (insurance billing, controlled-substance prescribing, multi-brand white-label) once V1 has validated demand.

02 PRODUCT VISION & GOALS

- [ ] Let a patient go from "I need care" to "a licensed clinician has seen my case" in under 10 minutes for async visits, or one scheduled video call for sync visits.
- [ ] Give clinicians a single queue-driven workspace to review intakes, run visits, document care, and prescribe — without juggling separate EHR, eRx, and video tools.
- [ ] Give Dr. Param's operations team full visibility and control: provider supply, compliance status, complaints, and revenue, in one console.
- [ ] Build on a HIPAA-eligible technical foundation from day one so V2/V3 features can be added without re-architecting.
- [ ] Ship a real, prescribable, billable V1 in approximately 17 weeks for a fixed investment of **$44,000**.

03 TARGET USERS & PERSONAS

Patient

Adult self-pay (and later insured) consumer looking for fast, convenient access to a specific service (weight loss, mental health, primary care).

Clinician / Provider

Licensed NP, PA, or physician contracted or employed by the platform, often working across multiple states.

Platform Admin / Ops

Dr. Param's internal team — operations, compliance, and support staff.

Need: See everything happening on the platform and intervene when something needs attention.

Need: Be seen quickly by a real, licensed clinician without visiting a clinic.

Need: Review cases and run visits efficiently inside one workspace; get paid correctly and on time.

04 VERSIONED SCOPE OVERVIEW

Version 1 — Launch MVP

~17 weeks · $44,000 fixed price

BUILD NOW

Goal: prove the core loop — discovery → intake → match → visit → prescription/care plan → payment — for one or two initial service lines, cash-pay only, sync video visits only.

PORTAL	IN SCOPE FOR V1
Patient App	Marketing landing page; account creation & identity verification; HIPAA + telehealth consent; condition-specific intake questionnaire (1–2 service lines); provider matching by state availability; sync video visit booking; visit summary & care plan; ePrescription hand-off to pharmacy; card payment via Stripe; rating prompt.
Provider Dashboard	Provider onboarding (license, NPI, DEA upload, e-signature); daily queue of scheduled visits; in-call dashboard with patient chart side-by-side; SOAP note charting; ICD-10/CPT code entry; eRx for non-controlled medications; availability & schedule management.
Admin Console	Patient & provider directory; manual provider approval; visit log & basic reporting (visits/day, revenue, conversion); flagged-case review; refund handling.

Version 2 — Scale & Monetize

~14-16 weeks · $58,000-$72,000 (planning estimate)

AFTER V1

Goal: remove the friction points that cap growth — add asynchronous visits, insurance, ongoing-care workflows, and native mobile.

PORTAL	ADDED IN V2
Patient App	Async (store-and-forward) visits; secure patient–provider messaging; medication refill requests; lab result delivery; subscription/package billing; insurance eligibility check & co-pay collection; native iOS & Android apps.
Provider Dashboard	Async case queue with approve/decline/request-info/refer actions; lab ordering; refill approval workflow; earnings & pay-stub view; license renewal tracking.
Admin Console	Automated provider credentialing workflow & background-check integration; multi-state license-to-patient-state matching; advanced reporting (cohort, retention, CAC); compliance monitoring dashboard.

Version 3 — Mature Platform

~12+ weeks or ongoing retainer · $75,000-$95,000

SCALE PHASE

Goal: the parts that matter once the platform has real volume — controlled substances, automation, interoperability, and white-label scale.

- [ ] AI-assisted symptom intake / triage; expanded service lines; loyalty / referral programs
- [ ] EPCS-compliant controlled-substance prescribing (two-factor DEA authentication)
- [ ] Direct lab integrations (Quest/LabCorp); productivity analytics
- [ ] Insurance claims automation (837/835, clearinghouse integration)
- [ ] SOC 2–ready audit infrastructure; white-label multi-brand configuration

05 TECHNICAL ARCHITECTURE PRINCIPLES

- **HIPAA-eligible from day one** — every PHI-touching service runs behind a signed BAA; retrofitting compliance later is far more expensive.
- **Data-driven configuration** — service lines, intake questionnaires, and state coverage are configured, not hard-coded, so adding a new condition or state in V2/V3 doesn't require new code.
- **Vendor stack (V1)** — Stripe (payments), DoseSpot or Doxy.me (eRx), video vendor TBD at discovery, Twilio or AWS Transcribe (comms), AWS/GCP with HIPAA BAA (hosting).
- **Shared data model** — Patient App, Provider Dashboard, and Admin Console all share one underlying patient/provider/visit data model, built once, used three ways.

06 RISK REGISTER

RISK	IMPACT	MITIGATION
Client delays BAA execution with PHI vendors	High	Hard dependency flagged in SOW; module development cannot begin until BAA confirmed
Initial service line or launch state not confirmed by Week 1	High	Provider matching and intake cannot be built without this; blocks M2
Scope creep via informal requests	Medium	All changes via signed change order; no out-of-scope items developed without written agreement
eRx vendor integration delays	Medium	Vendor selection finalised at Week 1; Hyperwrike has prior integration experience with DoseSpot
Clinician supply shortfall at launch	Medium	Client's responsibility to supply licensed clinicians; admin console built to handle small provider lists

07 INVESTMENT SUMMARY

$44,000

Version 1 — Fixed Price

~17 weeks

to go-live

Version 2 (planning estimate)	$58k–$72k
Version 3 (planning estimate)	$75k–$95k
V2 & V3 re-scoped at phase start	Not fixed yet

FOR **HYPERWRIKE**

AUTHORISED SIGNATORY

DATE

FOR **DR. PARAM**

AUTHORISED SIGNATORY

DATE